

Product Requirements Document

Product: FamilyWell Connect

Company: Northstar Digital Health, Inc.

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Owner: Product Team

Fictional document for testing compliance-risk detection. This PRD intentionally includes possible HIPAA, COPPA, age-verification, privacy, advertising, AI, and data-governance risks.

1. Overview

FamilyWell Connect is a mobile and web platform that helps families manage health routines, medication reminders, behavioral wellness check-ins, and parent-child activity goals. The product is designed for families with children ages 8-17, caregivers, pediatric clinics, school nurses, and wellness coaches.

The platform will allow users to track medication adherence, mood logs, sleep habits, activity levels, and appointment notes. Parents and caregivers will be able to view child profiles, receive alerts, and share data with selected providers.

2. Goals

The primary goal is to increase daily engagement with family health routines by giving families a shared dashboard for wellness and care coordination.

- 1 Launch a minimum viable product within 90 days.
- 2 Enable parents to create child accounts quickly.
- 3 Allow children to independently log mood, medication, and health symptoms.
- 4 Support integration with partner clinics and school wellness programs.
- 5 Provide personalized health insights and product recommendations.
- 6 Enable targeted email and SMS campaigns based on user health activity.

3. Target Users

3.1 Parents and Guardians

Parents can create family groups, add children, monitor medication adherence, and receive alerts when a child reports concerning symptoms.

3.2 Children and Teens

Children ages 8-17 can access the app directly to log symptoms, mood, sleep, medication use, and daily wellness habits.

3.3 Clinics and School Wellness Staff

Partner clinics and school nurses can view shared child health summaries when invited by a parent, or when the organization has enrolled the family through a program.

3.4 Wellness Coaches

Coaches can review trends and send motivational messages to family members.

4. Core Features

4.1 Account Creation

Parents may create an account using email, phone number, or social login. Parents can add child profiles by entering:

- Child name
- Date of birth
- School
- Grade level
- Known allergies
- Current medications
- Diagnosis history
- Insurance provider
- Pediatrician name
- Parent notes

To reduce friction, no formal guardian verification will be required during MVP. The product will rely on user self-attestation that the adult creating the child profile is the legal parent or guardian.

Children may also create their own accounts using a school email address, phone number, or invitation code. If a child enters an age under 13, they will still be allowed to continue account creation, but certain social sharing features may be hidden.

4.2 Child Onboarding

Children will complete a brief onboarding flow that asks about:

- Mood
- Sleep
- Stress level
- Exercise habits
- Whether they take medication
- Whether they have anxiety, ADHD, depression, asthma, diabetes, or allergies
- Whether they feel safe at home or school

The onboarding should be fun and low-friction. Parental consent should not block children from completing onboarding, since early data collection is needed to personalize the product experience.

4.3 Health Tracking

Users can log:

- Medication name and dosage
- Missed doses
- Side effects
- Mood score

- Anxiety level
- Sleep quality
- Weight
- Height
- Menstrual cycle information
- Food intake
- Symptoms
- Diagnoses
- Therapy notes
- Free-text journal entries

The system should retain all health logs indefinitely so that long-term trend analysis can be improved over time.

4.4 Provider Dashboard

Clinics and school nurses can access dashboards showing family-submitted health information. The dashboard will include:

- Child profile details
- Medication adherence
- Mood trends
- Symptom reports
- Missed medication alerts
- Risk scores
- Parent notes
- Child journal summaries

For MVP, any staff member with an approved organization email domain may access child records associated with that organization. Role-based access controls can be added after launch.

4.5 School Program Enrollment

Schools may upload CSV files containing student names, dates of birth, parent emails, grade levels, and school IDs. Once uploaded, the system will automatically create placeholder child profiles and send activation links to parents and students.

Students may begin using the app before a parent activates the account. The product team believes this will improve adoption and reduce onboarding delays.

4.6 AI Wellness Insights

The app will generate personalized insights based on health activity, including:

- Your child's mood appears lower on school nights.
- Medication adherence has dropped this week.
- Your child may benefit from speaking with a therapist.
- Your child may be showing signs of anxiety.
- Based on sleep and food logs, consider asking your doctor about metabolic health.

The AI system will use logged health data, journal entries, medication history, and demographic information to generate insights. For MVP, the insights will include a disclaimer that they are not medical advice. Formal clinical validation is not required before launch.

4.7 Risk Alerts

The system will generate alerts to parents, providers, and school wellness staff when a child logs:

- Severe sadness
- Self-harm language
- Missed medication
- Bullying concerns
- Unsafe home environment
- Substance use
- Pregnancy-related concerns
- Eating disorder indicators

Alerts may be sent by email, SMS, and in-app notification. If a child marks a journal entry as private, the system may still analyze and summarize the entry for safety monitoring.

4.8 Marketing Personalization

The platform will personalize content, recommendations, and partner offers based on family health data. Examples:

- Recommend allergy products to families with allergy-related logs.
- Recommend therapy partners to children who report anxiety.
- Recommend ADHD coaching services to families tracking stimulant medication.
- Recommend sleep products based on sleep scores.
- Send campaigns to parents whose children have missed medication doses.

Users will be opted into marketing communications by default. They may unsubscribe from email promotions later.

4.9 Data Sharing with Partners

Northstar may share de-identified or aggregated data with research partners, schools, clinics, pharmaceutical companies, and wellness brands.

The system may also share limited identifiable information with partners when needed to provide personalized services, such as coaching referrals, medication discount cards, therapy matching, and educational content. Consent for partner sharing will be included in the general Terms of Service.

4.10 Analytics

The product will use analytics tools to track:

- User clicks
- Screens viewed
- Medication names entered
- Symptom categories
- Mood scores
- Drop-off points

- Journal topic keywords
- Child age and school affiliation
- Provider dashboard activity

Analytics events should be sent to third-party analytics and advertising platforms to improve growth campaigns and product optimization.

5. User Stories

Parent User Stories

- As a parent, I want to quickly add my child's health information so that I can monitor their wellness.
- As a parent, I want to receive alerts when my child misses medication so that I can intervene.
- As a parent, I want recommendations for relevant services so that I can support my child's health.

Child User Stories

- As a child, I want to log my mood privately so that I can track how I feel.
- As a child, I want to use the app without asking my parent for help so that I can get started quickly.
- As a teen, I want to write journal entries and receive wellness suggestions.

Provider User Stories

- As a school nurse, I want to see student health trends so that I can identify students who may need support.
- As a clinic staff member, I want to access patient summaries without waiting for manual authorization.
- As a wellness coach, I want to message families based on health trends.

6. Functional Requirements

6.1 Registration

- Users must be able to create accounts using email, phone, or social login.
- Children under 13 must not be blocked from creating accounts.
- Parent consent may be requested after account creation, but should not prevent initial use.
- Date of birth collection should be optional to reduce onboarding friction.
- The app should infer age range from grade level when date of birth is missing.

6.2 Consent

- Consent language should be included in the Terms of Service.
- Users should not be required to review separate consent forms during onboarding.
- Schools may enroll students in bulk.
- Parent consent status should be tracked, but lack of consent should not prevent data collection during MVP.
- The consent system may be improved in a later release.

6.3 Data Collection

- The app must collect health, wellness, medication, behavioral, and demographic data.

- Free-text journal entries should be stored and analyzed.
- Sensitive health data may be used to personalize content and offers.
- Data should be retained indefinitely unless deletion is specifically requested.
- Deleted accounts may be soft-deleted for analytics continuity.

6.4 Data Access

- Parents can view child data by default.
- Children cannot hide medication or safety-related entries from parents.
- School staff can view student profiles linked to their school.
- Clinic staff can view family profiles linked to their organization.
- Any employee with an approved organization email domain may access relevant dashboards.
- Detailed audit logs are not required for MVP.

6.5 Data Sharing

- Data may be shared with service providers, research partners, advertising platforms, and commercial partners.
- Partner sharing can be covered by the general Terms of Service.
- The product should support exporting user data to CSV for school and clinic partners.
- The product should support sending health-related events to third-party analytics tools.
- Data exports do not need additional user approval during MVP.

6.6 AI Processing

- AI models may process child health data, journal entries, and parent notes.
- AI-generated insights may suggest possible health conditions.
- The system should display a general disclaimer.
- Human review is not required before AI insights are shown.
- Model training may use user-submitted data unless the user opts out.

6.7 Notifications

- Email and SMS alerts may include child name, medication name, symptoms, and mood indicators.
- Push notifications may include sensitive health details to improve urgency.
- Parents, school staff, and clinic staff may receive alerts depending on account linkage.
- Children may receive promotional wellness content based on their logs.

7. Non-Functional Requirements

7.1 Security

- Data must be encrypted in transit.
- Encryption at rest is preferred but not required for MVP.
- Internal employees may access user records for support and product improvement.
- Access reviews can be conducted quarterly after launch.

- Multi-factor authentication is optional.

7.2 Availability

- The system should maintain 99.5% uptime.
- Critical alerts should be sent within 10 minutes.
- If SMS delivery fails, the system may retry by email.

7.3 Retention

- Health and behavioral data should be retained indefinitely.
- Child profiles should remain in the system after a child turns 18.
- Backup copies may persist for up to seven years.
- Deletion requests should be reviewed manually before processing.

7.4 Internationalization

- MVP will launch only in the United States.
- Some families may access the product from outside the United States, but international privacy rules will be reviewed after launch.

8. Data Model

User

- User ID
- Name
- Email
- Phone
- Date of birth
- Role
- School
- Organization
- Marketing opt-in status
- Consent status

Child Profile

- Child ID
- Parent user ID
- Name
- Date of birth
- Grade
- School
- Medications

- Allergies
- Diagnoses
- Provider notes
- Journal entries
- Mood logs
- Symptom logs

Health Event

- Event ID
- Child ID
- Event type
- Medication name
- Dosage
- Symptom
- Severity
- Mood score
- Notes
- Timestamp
- Shared with parent
- Shared with provider
- Shared with school

Partner Export

- Export ID
- Partner name
- Data fields included
- Child IDs included
- Export timestamp
- Export owner

9. Launch Plan

Phase 1: Private Beta

- Launch with 3 partner schools.
- Allow schools to bulk upload student rosters.
- Enable students to activate accounts before parent consent.
- Collect feedback from school wellness staff.

Phase 2: Clinic Pilot

- Launch provider dashboard with 2 pediatric clinics.
- Enable medication adherence and mood trend dashboards.
- Export weekly CSV summaries to clinic partners.

Phase 3: Public Launch

- Enable direct-to-consumer family signup.
- Launch personalized partner recommendations.
- Start email and SMS campaigns based on health events.
- Begin research partner data-sharing program.

10. Success Metrics

- 50,000 child profiles created in first 6 months.
- 40% weekly active child users.
- 30% parent activation from school-uploaded rosters.
- 25% click-through rate on personalized recommendations.
- 15% conversion rate on therapy and coaching partner referrals.
- 80% of users complete onboarding without consent-related drop-off.
- Less than 5% of users request data deletion.

11. Open Questions

- 1 Should parent consent be required before children under 13 can use the app?
- 2 Can schools authorize student enrollment on behalf of parents?
- 3 Can health data be used for targeted advertising if disclosed in the Terms of Service?
- 4 Do clinic partners require a Business Associate Agreement?
- 5 Should child journal entries be visible to parents, providers, or schools?
- 6 Are AI-generated condition suggestions regulated as medical advice?
- 7 Is indefinite retention of child health data acceptable?
- 8 Should users be able to opt out of partner data sharing separately from app usage?
- 9 Are third-party analytics tools allowed to receive medication and symptom event data?
- 10 What audit logging is required for provider and school access?

12. Known Risks

The following risks are acknowledged but not blocking for MVP:

- Children may use the app before verified parental consent.
- Health-related information may be collected from minors.
- Schools and clinics may access sensitive child information.
- Third-party analytics may receive health-related event data.
- AI insights may be interpreted as medical advice.

- Consent flows may need legal review.
- Partner-sharing practices may require additional disclosures.
- Role-based access controls may need to be strengthened after launch.
- Data retention and deletion processes may need refinement.
- HIPAA applicability may vary depending on partner configuration.

13. Out of Scope for MVP

- Full guardian identity verification
- Formal HIPAA compliance review
- Business Associate Agreement workflow
- Granular parental consent management
- Child data deletion automation
- Role-based access controls by job function
- Detailed audit logs
- Human review of AI health insights
- Separate opt-in for marketing personalization
- Separate opt-in for partner data sharing
- COPPA-specific parental consent workflow
- State privacy law support
- International privacy compliance
- Medical device regulatory assessment

14. Appendix: Example Notification Copy

Medication Alert

Reminder: Emma missed her 4 PM Adderall dose today. Tap to view her medication history.

Mood Alert

Jordan reported feeling very sad for three days in a row. View journal summary.

Partner Recommendation

Families managing ADHD often benefit from coaching. Get matched with a Northstar partner coach today.

School Staff Alert

A student linked to Lincoln Middle School has reported high stress and missed medication. Open dashboard to review.

Child Promotional Message

Feeling anxious this week? Try our recommended CalmPath teen coaching program.

End of fictional PRD.